Gefitinib monotherapy

> **Which treatment this guide covers** NCCP 00220a covers adults with locally advanced or metastatic non-small-cell lung cancer (NSCLC) with an activating EGFR mutation. This is continuous treatment for advanced disease, not a fixed adjuvant course.
>
> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Gefitinib alone · daily oral treatment

Active medicines: gefitinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.
- Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.
- Gefitinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.
- Give pharmacy all medicines and supplements. Strong CYP3A4 inducers can reduce exposure; inhibitors and some CYP2D6 interactions can increase risk. Medicines that keep stomach acid low may reduce absorption. Ask for an individual medicine plan; do not borrow timing from another EGFR medicine. Warfarin needs regular blood-clotting checks.
- Avoid pregnancy during treatment and discuss effective contraception and future pregnancy plans with the team. National Cancer Control Programme (NCCP) excludes pregnancy and breastfeeding; do not breastfeed during treatment. The cited product information does not specify a universal post-treatment contraception interval, so ask for an individual plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00220 v5 · p3 · NCCP V6 oral minimal-low: individual as-required antiemetics; NCIS V8 is parenteral and does not prescribe an oral course

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required